

PM-JAY Uttar Pradesh -- Programme Overview

Key patterns and structural findings across the scheme

Claims Processing & Treatment Patterns

Claims value is concentrated in Cardiology and Orthopaedics

Across Uttar Pradesh, the value of claims is heavily concentrated in a small number of specialties. Cardiology and Orthopaedics together dominate the claim ledger, and this pattern is consistent across all 18 divisions.

- Cardiology: INR 40.19 crore claimed, or 39.4% of total claimed value
- Orthopaedics: INR 26.02 crore claimed, or 25.5% of total claimed value
- Together, Cardiology and Orthopaedics account for 64.9% of all claim amounts
- Next highest specialties are Obstetrics & Gynaecology at INR 8.50 crore and General Surgery at INR 7.22 crore
- The same concentration appears in standard package pricing: Cardiology is 42.7% of total base amount, followed by Orthopaedics at 24.1%

Older patients and chronic/surgical care drive most package value

The highest package values are associated with older age groups, especially patients aged 41 and above. This suggests that PM-JAY spending is being pulled by chronic disease and higher-complexity treatment needs rather than younger-age case loads.

- Patients aged 60+ account for INR 3.36 crore of base amount, or 37.2%
- Patients aged 41-60 account for INR 2.99 crore, or 33.1%
- Together, the 41+ age groups make up 70.3% of total base amount
- Patients aged 26-40 account for INR 1.89 crore, or 20.9%
- Patients aged 15-25 account for INR 80.27 lakh, or 8.9%

Most admissions end normally, and this pattern is uniform across divisions

Patient outcomes are largely stable: most cases end in a normal discharge, with very small shares ending in death or against-medical-advice discharge. This discharge pattern is consistent across all 18 divisions.

- Normal discharge accounts for 19,650 cases, or 87.3% of all admissions

- Live/ongoing cases account for 1,403 cases, or 6.2%
- LAMA cases account for 891 cases, or 4.0%
- DAMA cases account for 470 cases, or 2.1%
- Deaths account for 86 cases, or 0.4%

Claim processing is mostly approved, but a meaningful share still sits in query or rejected status

On the financial processing side, most claimed value is approved, but not all of it has translated into payment yet. There is still a noticeable pipeline of queried, rejected, and pending amounts, indicating that claims management remains active.

- Approved claims account for INR 64.69 crore, or 72.2% of claimed amount
- Settled claims account for INR 10.96 crore, or 12.2%
- Query-raised claims account for INR 8.31 crore, or 9.3%
- Rejected claims account for INR 3.60 crore, or 4.0%
- Pending claims account for INR 2.03 crore, or 2.3%

Settlement time is fairly even across districts, with modest variation

Among settled claims, claim payment turnaround is broadly similar from district to district. The spread is not wide, suggesting relatively even settlement performance across the state, though some districts are still slower than others.

- Average settlement time across districts is 938.98 days in the dataset summary measure
- Highest district average is Bahraich at 15.09 days
- Basti follows at 14.79 days, and Lalitpur at 14.75 days
- Pilibhit averages 14.44 days and Jhansi 14.34 days
- Lowest district averages are Auriya at 9.28 days and Maharajganj at 9.94 days

Admissions show a clear seasonal pattern, with a sharp dip in February 2023

Case volumes rise and fall in a repeating yearly pattern, with the highest volumes in mid-year months. One month stands out as an outlier, with admissions far below the usual level.

- Highest monthly case volume is 818 cases in August 2023
- August 2025 recorded 777 cases, July 2023 recorded 776, and July 2024 recorded 767
- July 2025 still remained high at 748 cases
- The lowest month is February 2023 with only 20 cases
- January 2026 stands at 544 cases, below the peak months but far above the February 2023 low

District-Level Monthly Performance Trends

Beneficiary enrolment peaked in 2023 and then tapered off

Across Uttar Pradesh, new beneficiary enrolment was strongest in 2023 and then fell away in the later years of the period. This pattern is visible at the district level and also when the data is viewed by month, quarter, or year, suggesting a broad statewide slowdown after the main enrolment push.

- 2023 recorded **88,591** new beneficiaries, the highest annual total, equal to **43.0%** of all new enrolments
- 2024 still contributed **57,973** new beneficiaries, or **28.2%** of the total
- 2022 added **46,152** new beneficiaries, or **22.4%** of the total
- 2025 fell to **13,129** new beneficiaries, just **6.4%** of the total
- Monthly enrolment was highest in **2023-05** at **7,690**, with other high months at **7,683** in **2023-07** and **7,658** in **2023-03**
- The monthly pattern was judged **decreasing in all 18 divisions**, with no division showing a different direction

Household enrolment followed the same statewide timing

New household enrolment moved in step with beneficiary enrolment. The annual split is almost identical, with a strong concentration in 2023 and a much smaller contribution from 2025, which indicates that the main enrolment drive was concentrated in the middle of the period.

- 2023 accounted for **83,025** new households, or **42.5%** of the total
- 2024 contributed **55,394** new households, or **28.4%**
- 2022 contributed **43,991** new households, or **22.5%**
- 2025 contributed **12,918** new households, or **6.6%**
- The monthly series shows the same broad seasonal pattern as beneficiary enrolment, with a common peak period across the state
- This pattern was consistent across **all 75 districts**, with no district-level exceptions reported

Claims activity rose after an early 2023 turning point, with clear seasonality

Hospital admissions under PM-JAY shifted around **2023-03**, after which claims activity generally moved into a higher operating range. The monthly pattern is not flat: claims repeatedly rise in the middle of the year, especially around the same months, which suggests a recurring seasonal cycle in utilisation.

- A change point is visible at **2023-03** for claims admitted across **all 18 divisions**
- The highest monthly admissions were **818** in **2023-08**
- Other high months were **777** in **2025-08**, **776** in **2023-07**, **767** in **2024-07**, and **748** in **2025-07**
- The monthly admissions total across the period was **22,500**

- At the quarter level, the statewide shift was also marked at **2023Q1**
- One district, **Balrampur**, differed slightly from the main quarterly turning point and showed the highlighted change in **2023Q2** instead

Beneficiary coverage kept rising, but quarterly gains were strongest in the latest period

The cumulative beneficiary base continued to expand steadily throughout the period, which is what you would expect in a mature insurance programme. When viewed by quarter, the average cumulative count is highest in the most recent quarters, showing that enrolment has kept building even as fresh monthly enrolment slowed.

- In annual terms, cumulative beneficiaries reached **202,847** by the end of the period
- The largest annual contribution came in **2023** with **88,591** beneficiaries, or **43.0%** of the total cumulative base counted in the summary
- **2024** added **57,973** beneficiaries, or **28.2%**
- **2022** contributed **46,152** beneficiaries, or **22.4%**
- The quarterly average cumulative beneficiary count rose to **2,950.14** in **2026Q1**, compared with **2,750.54** in **2025Q4**
- The lowest quarterly average was **15.51** in **2022Q1**, showing how much the base expanded later in the period

Utilisation and financial processing were broadly stable across districts, with modest variation

Claims utilisation, approval behaviour, and payment flow appear fairly even across the state, with no major district outliers in the summary view. The overall district averages are close together, and the reported payment and claim measures also show a common seasonal pattern rather than fragmented district behaviour.

- Average claims per **1,000 beneficiaries** ranged from **2.59** in **Hamirpur** to **3.79** in **Amroha**
- Other high-use districts included **Mathura** at **3.66**, **Kasganj** at **3.61**, **Siddharthnagar** at **3.60**, and **Sultanpur** at **3.54**
- The lowest reported value among the listed districts was **2.66** in **Sant Ravidas Nagar**
- The quarterly claims-per-1,000 series changed at **2023Q1**, with the highlighted value at **3.2**
- The monthly pattern across measures shows a common seasonal period for **20 of 26** tracked measures
- Payment-related measures and claims processing measures also show a common shift around **2023-03**, indicating that utilisation and financial flow moved together

Hospital Infrastructure & Specialty Capacity

Spending is concentrated in Cardiology and Orthopaedics

PM-JAY claim spending across empanelled hospitals in Uttar Pradesh is heavily concentrated in two specialties: Cardiology and Orthopaedics. Together, they account for almost two-thirds of all claimed amount, and this pattern is consistent across all 18 divisions in the state.

- Cardiology: INR 40.04 crore claimed, 39.9% of the total INR 100.27 crore
- Orthopaedics: INR 25.78 crore claimed, 25.7% of the total
- Next largest specialties: OBG at INR 8.49 crore and General Surgery at INR 7.22 crore
- Urology is a smaller contributor at INR 4.52 crore, or 4.5%
- Burns and Psychiatry recorded INR 0 claimed

Larger hospitals carry most of the bed capacity

Hospital bed strength is concentrated in larger facilities. Across all specialties, hospitals in the 101-300 bed category account for the largest share of total bed strength, while very small hospitals contribute only a small fraction.

- Large hospitals (101-300 beds): 594,988 beds, 62.9% of total bed strength
- Very large hospitals (300+ beds): 163,126 beds, 17.2%
- Medium hospitals (31-100 beds): 160,335 beds, 16.9%
- Small hospitals (30 beds or fewer): 27,537 beds, 2.9%
- In public hospitals alone, large facilities still lead: 189,149 beds, 38.9% of the public-hospital total
- In public hospitals, small facilities contribute 20,872 beds, only 4.3%

Staff and licences are fairly evenly distributed across hospital types

Clinical staffing levels and licence holdings look broadly even across hospital sub-types and bed-size groups. Average staff strength per specialty is close across categories, and average licence counts in public hospitals vary only slightly by hospital size.

- Average total staff per specialty is very similar across hospital sub-types, ranging from 5.13 to 5.52
- Dermatology has the highest average staff at 5.52, followed by Psychiatry at 5.50 and Burns at 5.45
- Cardiology has the lowest average staff among the top end at 5.13
- In public hospitals, average total licences are almost flat across size groups: 7.15 for small, 7.05 for medium, 7.01 for large, and 6.94 for very large
- Average staff experience is also steady across divisions, from 16.98 years in Jhansi to 19.71 years in Chitrakoot
- Lucknow's average staff experience is 19.49 years, and Gorakhpur's is 18.96 years

Case volume per bed is highest in smaller public hospitals

Utilisation, measured as PM-JAY cases per bed, is highest in smaller public hospitals and drops as hospital size increases. This pattern is seen consistently across all 18 divisions, showing that smaller public facilities are handling more cases per bed than larger ones.

- Public small hospitals (30 beds or fewer): 0.22 cases per bed on average
- Public medium hospitals (31-100 beds): 0.05 cases per bed
- Public large hospitals (101-300 beds): 0.02 cases per bed
- Public very large hospitals (300+ beds): 0.01 cases per bed
- Small public hospitals account for 73.9% of the total cases-per-bed distribution
- The same pattern holds across all 18 divisions with no exceptions

Treatment activity is uneven across specialties, with some gaps in lower-volume areas

Across treatment and approval measures, the largest specialties are mostly the same, but there are clear gaps in low-volume specialties. Burns and Psychiatry repeatedly sit at the bottom, and some specialty-by-hospital combinations had no claims at all.

- In the admissions pattern, large hospitals lead with 2,848,517 admissions before last year, 41.3% of the total
- Medium hospitals follow with 2,266,222 admissions, 32.9%
- Small hospitals account for 1,347,505 admissions, 19.5%
- Very large hospitals have the lowest share of admissions before last year at 431,586, or 6.3%
- In the admissions pattern, Dermatology and Psychiatry are the two exceptions to the main trend
- For cases per bed by specialty, Burns and Dermatology both sit at 0.0 in the reported pattern, while Obstetrics and Gynaecology leads at 0.1

Beneficiary Enrolment & Scheme Uptake

Enrolment documentation is broadly consistent, with small district-level variation

The enrolment file looks fairly uniform across Uttar Pradesh on the basic documentation metric. Average document count is almost the same across divisions, which suggests enrolment teams are working to a similar standard rather than showing large local differences.

- Average document count is 1.21 in Devipatan and Saharanpur, the highest reported among divisions
- Kanpur, Basti, and Ayodhya are at 1.20 documents on average

- Meerut and Aligarh are the lowest reported at 1.19 documents on average
- The division-level spread is very narrow: 1.19 to 1.21 documents per beneficiary
- The measure is even across all 4 age groups: 15–25, 26–40, 41–60, and 60+

Claims are concentrated among older beneficiaries, with SECC driving most utilisation

Scheme uptake is clearly stronger in the older age groups. Beneficiaries aged 60+ generate the largest share of claims, while the 15–25 age group contributes the least. Across entitlement sources, SECC beneficiaries account for the majority of claim activity, which means the original census-linked group remains the main driver of utilisation.

- Beneficiaries aged 60+ filed 8,649 claims, or 38.4% of all claims
- The 41–60 group filed 6,730 claims, or 29.9%
- The 26–40 group filed 5,093 claims, or 22.6%
- The 15–25 group filed 2,028 claims, or 9.0%
- SECC beneficiaries accounted for 13,547 claims, or 60.2% of all claims
- STATE_DB contributed 5,630 claims (25.0%), and RSBY contributed 3,323 claims (14.8%)

Active beneficiaries with valid records are far more likely to use the scheme, and gold records lead the way

Among active cardholders, verified records are the strongest source of utilisation. GOLD records contribute the largest share of claims, with SILVER and PENDING trailing well behind. The same age pattern repeats here: older active beneficiaries use the scheme more than younger ones.

- Among active cardholders, GOLD records generated 11,089 claims, or 59.5%
- SILVER records generated 5,648 claims, or 30.3%
- PENDING records generated 1,906 claims, or 10.2%
- In active cards, beneficiaries aged 60+ filed 7,178 claims, or 38.5%
- The 41–60 group filed 5,622 claims, or 30.2%
- The 15–25 group remained the smallest contributor at 1,667 claims, or 8.9%

Card issuance and Aadhaar linkage look stable across the state

The operational side of enrolment appears steady across districts and entitlement sources. Aadhaar linkage among active beneficiaries is very similar across divisions, and card issuance timing is also consistent across districts and entitlement groups. This points to a uniform operational process rather than a few districts moving much faster than others.

- Active beneficiaries have Aadhaar linkage averaging 0.82 in Hamirpur, Firozabad, Badaun, Maharajganj, and Etawah
- The lowest reported Aadhaar linkage is 0.78 in Gautam Buddha Nagar and Mahoba
- The district-level spread for Aadhaar linkage among active beneficiaries is only 0.78 to 0.82

- Average days from enrolment to card issuance are 15.49 days for SECC beneficiaries
- STATE_DB beneficiaries average 15.48 days from enrolment to card issuance
- RSBY beneficiaries average 15.44 days from enrolment to card issuance

Claim rates are low overall, but virtually identical across entitlement sources

Even though claim volumes are meaningful in absolute terms, the share of enrolled beneficiaries who have used the scheme remains low overall. The claim rate is remarkably even across age groups, which suggests the basic probability of using PM-JAY is not being shaped strongly by age at the all-state level. It is also nearly identical across entitlement sources.

- Claim rate is about 0.1 across all age groups: 15–25, 26–40, 41–60, and 60+
 - The state has 205,847 enrolled beneficiaries in total
 - Across entitlement sources, claim rate is evenly distributed across SECC, STATE_DB, and RSBY
 - The age-group sample sizes behind the claim-rate view are 18,484 for 15–25, 46,152 for 26–40, 61,457 for 41–60, and 79,754 for 60+
 - Card-to-first-claim timing is grouped with the other evenly distributed operational measures across the state
-